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"AR Billing Matrix Standardization (B.M.S) Project of Account Receivables Department at DM Clinical Research."

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"BILLING MATRIX STANDARDIZATION PROJECT OF ACCOUNT RECEIVABLES DEPARTMENT AT DM CLINICAL RESEARCH"

1 ABSTRACT:

This "Project Initiation Document" (PID) describes how "DM Clinical Research" (DMCR) would establish and execute a standardized billing matrix for "Accounts Receivable" (AR) billing items in accordance with its "Clinical Trail Agreement" (CTA) signed with the trial Sponsors. DMCR is one of the largest US private clinical trial networks, providing high-quality clinical research for 20 years in essential therapeutic domains.DM has 15 different sites. They help with patient recruiting, trial coordination, and site management for prominent worldwide sponsors like "Pfizer, Moderna, and AstraZeneca." (DMCR, 2025) DM Clinical Research conducted this study to address the discrepancies in the CTA's site-level expense line items concerns. The DM accounts receivable project management office implemented a fixed billing matrix method due to invoice rejections, audit anomalies, and payment delays. The project used PRINCE2 principles to construct a centralized and uniform billing matrix to emphasize the need for a clear business rationale for all project activities and clearly defined roles and obligations. (Susan Tuttle, 2018) The project used PMBOK Guide Knowledge Areas such regulating scope, cost, and quality (Project Management Institute, 2021) to ensure a methodical approach. This led to a reduction in financial uncertainty, enhancement of interdepartmental communication, improved audit preparation, and increased cash flow efficiency. The standardized matrix enhanced the AR department's financial reconciliation processes and DMCR's sponsor relationships by ensuring clarity, professionalism, and regulatory compliance. DMCR has become a more open, reliable, and fiscally responsible research partner. Subsequent parts will examine the project's implementation, achievements, and challenges, encompassing regulatory constraints and opposition to interdepartmental change. This case study provides instruction on prospective enhancements in financial processes using valuable project management tools, principles and techniques for managing a Q1 level tradition project.

CTA Billing Matrix, PRINCE2, PMBOK, Clinical Trials, AR Compliance, Financial Standardization.

2 PROJECT DEFINATION:

2.1 PROJECT BACKGROUND

This project aims to instate a reusable standardize billing matrix for all the site level incurred costs within **DM Clinical Research's ("Institution") Account Receivable (AR)** department for all the ongoing **Modern Pharmaceutical ("Sponsor")** clinical trials, in accordance to the **Clinical Trial Agreement CTA**. Currently, the institution is facing inconsistent billing interpretation of the site level AR cost budgeted on the CTA, that is agreed between DMCR (Institution) and Modern Pharmaceutical ("Sponsor") with the governances of the **Institutional Review Board (IRB)**, CTAs differ from conventional commercial services agreements since they emphasize trial protocols and risk management, rather than have a formal business or financial language. (Pfeiffer & Frances, 2015) This can pose a threat to the agreed terms between both the parties for the things that are tied to finance and can be interpreted later on once the trail starts and the costs are incurred. As the CTA has these undefined financial terms and conditions referring to them as "grey area" or vague language which leads to cause invoicing errors, financial compliance risks, audit discrepancies, invoice rejections and payment delays. (Rijswijk, et al., 2012) Based on these highlighted issues. The proposed project to install a standardized billing matrix for these clinical trial agreement will introduce a uniform matrix template for the institute's account receivable department to improve its billing processes, invoice compliance, reconciliation rate, payment delay and unpaid revenue aging that will ensure alignment between DMCR and Sponsor finical review dashboard.

With context to the clinical industry, a clinical research institution work with sponsors who agree on billing schedules through CTAs. However, each CTA varies in structure and language, leading to diverse interpretations and inconsistent billing practices. (Parke, 2013) This creates compliance challenges, missed revenue, and inefficiencies across departments, moreover any change within the organization should be implemented in a systematic way through projects, programs and portfolios in order to deliver a valuable clinical trial for any clinical research company. (Project Management Institute, 2021) This starts a chain of commands within the organization to deliver these projects by cross communicating with all the stakeholders involved in making this clinical trial a success.

2.2 PROJECT OBJECTIVE

The proposed project implemented a standardized matrix template for the institution and conveyed the process to the sponsor concerning the financial terms of the clinical trials agreement related to the site-level budget discrepancies. As this project seeks to achieve financial compliance in the AR department and ensure alignment among institutional stakeholders, including Accounts Receivables (AR), Accounts Payables (AP), Clinical Operations (Sysops), Contracts and Budgeting (Contracts) and the external stakeholders such as Institutional Review Board (IRB), Sponsors and vendors. This will reduce billing inconsistencies and improve audit preparedness. Establishing a systematic procedure for institutional departments to implement or adapt to any modifications to the Clinical Trial Agreement from the sponsor or the Institutional Review Board committee.

2.3 PROJECT SCOPE

This project under AR department at DMCR amid for creating and presenting a standardized CTA billing matrix to the executive board within a six-month timeframe, as well as interpreting and categorizing site-level accounts receivable billing items across all clinical trial agreements within the institution, which were then documented and processed into the system with the assistance of the system operations team. This matrix will serve as a consolidated reference for synchronizing billing methods among project partners. (Shane & Dennis, 2021) Internal stakeholders include Accounts Receivables (AR), Accounts Payables (AP), Clinical Operations (Sysops), and Contracts and Budgeting (Contracts). External stakeholders comprise the Institutional Review Board, third-party contractors, sponsors and patients.

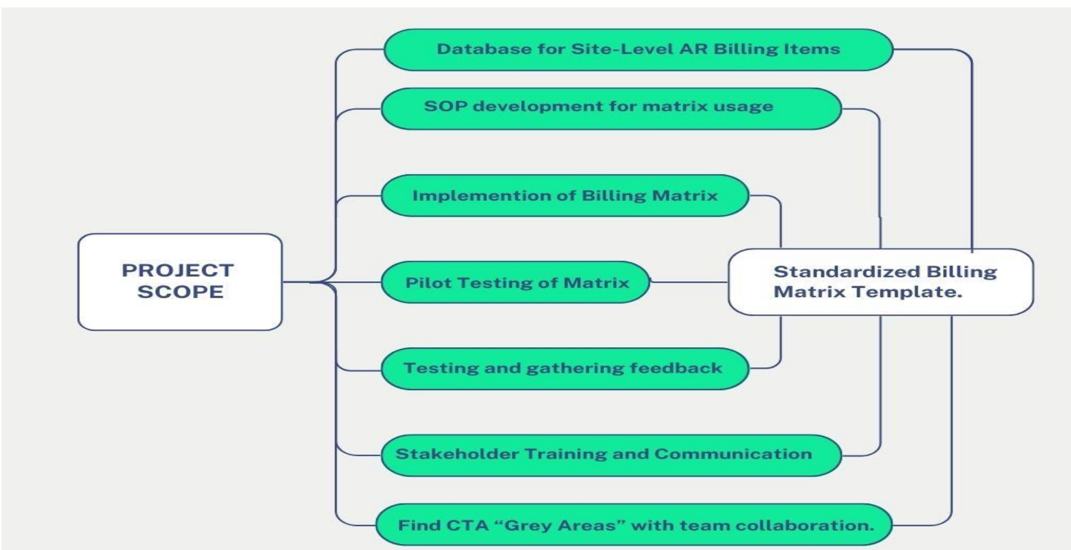


Figure:1 (Elements of project scope for standardized billing matrix template (personal Collection,2025)

2.4 PROJECT EXCLUSIONS

The project exclusion criteria clearly specify the tasks and responsibilities that were not included within the project scope. These aspects impacted project execution or post-implementation, however these remained unaddressed within the project scope, here are the tasks that were out of project scope: any contractual or financial negotiation on CTA between DM and sponsors or vendors, the development of clinical trial administration software, billing processes which include collections or invoicing, resolution of legal or regulatory disputes for CTAs, filling of legal approvals and, extended training programs beyond pilot phase.

2.5 PROJECT CONSTRAINTS

The project management office at DM encountered multiple obstacles concerning the delivery and implementation of an effective AR billing matrix. Some of the most prominent constrains were as follows:

Constraint	Impact	Likelihood
Regulatory Compliance Issues	High - May result in regulatory non-compliance and penalties	Medium
Time Constraints	Medium - Slows down approval and adoption process	High
Budget Limitations	Medium - May limit tool choice or external consultations	Medium
Limited Human Resources	Medium - Slow development, training, and adoption	Medium
Stakeholder Misalignment	Medium - Slows down approval and adoption process	Medium
Online and Remote Setting	Teams collaboration affected by poor internet connectivity.	High

Table:1 (DM Billing Metric Standardization Project's Constraints)

2.6 Project Assumptions

During the project's execution, expectations and assumptions regarding the ultimate outcome were established, these assumptions were tracked by the PMO team, which were documented in the assumption log for tracking the project's overall success. Here are the assumptions that were logged by the project team.

Assumption no.	Description
A01	CTA language can be standardized across trials and sponsors
A02	Internal stakeholders will implement and utilize the new billing matrix.
A03	The Matrix will seamlessly interact with existing DMCR financial system.
A04	Sponsors will endorse the standardized billing structure.
A05	Support from executive level and adequate resource will be provided for the project
A06	DMCR's historical CTA and billing data is precise and readily available.
A07	Project has a realist time frame and can be completed within 6 months.

Table:2 (DM Billing Metric Standardization Project's Assumptions)

2.7 PROJECT GOVERNANCE

The project management office at DM has adopted a decentralized governance model to facilitate efficient project execution. This model has been developed and validated by the research of Zhao and Qiu, focusing on a decentralized governance framework appropriate for the execution of entirely remote projects within an organization, in alignment with each team's accountability. (Zhao & Qiu, 2025) This model not only illustrates the definitions for digital responsibility but also underscores the ongoing issues an organization faces in data management, communication planning, and the orchestration of digital project governance techniques. (Bharadwaj, et al., 2013) The geographical separation of the AR department, contracts, and budgets at DMCR in Pakistan from the execution leadership and system operation teams in Houston, Texas, posed significant challenges in establishing effective project governance. Thus the DRSG framework, created by Zhao and Qiu, is optimal as it was specifically designed for startups employing digital technologies with decentralized governance models, which aligned perfectly with DM Clinical Research's organizational structure, as they are spread across different time zones and this governance model can work irrelevant of such challenge. Although approaches such as SCRUM are appropriate for IT-centric product development, whereas Digital Responsibility for Startups' Governance is better suited for a financial project governance model, that can easily incorporate stakeholder's accountability across task management and project execution. (Zhao & Qiu, 2025)

3 PROJECT CHARTER

Project Charter

Project General Information

Project Title	Billing matrix standardization project of account receivables department at DM Clinical Research.
Project Sponsor	DM Clinical Research Executive Board.
Project Manager	Accounts Receivable Compliance Administrative Team.
Project Duration	June 2024 – November 2024 (Six months)

Project Goal/Objective

The ultimate goal and objective of this project was to resolve all the finance related discrepancies and deliver a successful standardizes billing matrix for the approved budget in clinical trial agreement in between DM clinical research and the trial sponsor.

Project Scope

Within Scope: Analysis of current accrual processes. Development of a standardized accrual model/template Analysis of current accrual processes. Development of a standardized accrual model/template. Installing a cross-departmental communication plan. Training sessions for finance and clinical operations teams. Post-implementation review and adjustment

Out of Scope: Any CTA contract or financial agreement between DM and sponsors or vendors. Software development and integration for the billing system to the DMCR clinical trial management system. Collected or invoiced billing. Legal approvals and CTA dispute resolution. Further training after pilot.

Project Deliverables

- DMCR Site-Level Accrual Standard Template has been finalized.
- Delivery of Standard Operating Procedures for the newly build billing matrix.
- Initial training and workshop.
- Guide for System Administrator Integration.
- Post-project evaluation and insights gained.

Project Risks

- Regulatory Compliance Issues
- Time Constraints
- Budget Limitations
- Limited Human Resources
- Stakeholder Misalignment
- Online and Remote Setting

Project Assumptions

CTA language can be standardized across trials and sponsors, Internal stakeholders will implement and utilize the new billing matrix, the Matrix will seamlessly interact with existing DMCR financial system. Sponsors will endorse the standardized billing structure. DMCR's historical CTA and billing data is precise and readily available. Project has a realist time frame and can be completed within 6 months.

Project Budget

Total estimated budget projection $320,000 Inclusive of (Core Team, recruitment, outsourcing of professional project tools, Contingency Buffer)

Project Timeline	Requested Approval
○ **[Project Initiation Date**: 25th June 2024] ○ **[Project Completion Date:** 30th November 2024] ○ **[Sponsor and IRB submission for approval**: 1st December 2024] ○ **[Training and implementation**: 1st January 2025]	✓ **[Project Approved by]** Chief Finance Officer of DM Clinical Research (CFO) ✓ [Approval request Date: 22nd Feb 2024] ✓ [Project Approval received: 15th May 2024] ✓ [Project Proposed by] AR Compliance Manager: M Wali Baig ✓ [Project Management Office] Finance Department

Exhibit 1: (DM Billing Metric Standardization Project Charter) (Personal Collection, 2025)

4 KPROJECT APPROCH:

4.1 Execution Methodology

The DMCR standardized billing matrix project was managed via adoption of a hybrid approach. "Customizing the development methodology, life cycle, and processes to align with the project's attributes is a fundamental practice. A hybrid method enables a project to utilize the advantages of both predictive and adaptive strategies in their optimal contexts. (Project Management Institute, 2021).

The AR project manager advised this hybrid method following an analysis of the project team's capabilities and the project tasks. This approach was aligned with the project management institute's standards, The Predictive (Waterfall) components provided the financial processes, compliance, and audit readiness with the necessary structure, documentation, and control. This ensured that outputs, such as standardized templates and SOPs, adhered to the appropriate standards.

Simultaneously, Agile principles were employed during the design, testing, and stakeholder engagement phases to enhance flexibility, cooperation, and iterative feedback loops. These were crucial for securing internal team support and enhancing the solution for practical use.

4.2 Project Stage Plan:

The project lifecycle was broken down into five unambiguous phases, combining the stability of a predictive model with the flexibility of agile methods:

Project Life Stages	Associated Tasks	Associated Tools	Selected Approach
Project Initiation Stage	Project plan and charter approval, distinguishing stakeholders (internal & external).	Project Charter, WBS and KPIs, Jira.	**Predictive**
Project Planning Stage	Defining the scope in detail, managing risks, planning resources (historical data collection, personnel management), and making a communication plan and project timeline.	Project Road Map, Project Plan, Resource Plan, Communication Plan.	**Predictive**
Project Execution Stage	Building matrix and setting standards, conducting stakeholder workshops, and performing integration testing.	Kanban Board, Gantt Chart, Clinical Financial Dashboard. Power BI	**Hybrid (Agile & Predictive)**
Project Controlling Stage	Progress tracking, risk reviews, quality checks, stakeholder feedback cycles (in loop) for re-testing, delivering project process report to stakeholders. Finalizing amendments to project matrix.	Google Survey, Email, Tableau, Sprint. Google Teams	**Hybrid (Agile & Predictive)**
Project Closure	Final delivery of the Standard Operating Procedure, training sessions, insights gained, and formal approval.	Email Google Teams Review Report	**Predictive**

Table 3: (DM Billing Metric Standardization Project's Life Stages) (Meredith & Shafer, 2022)

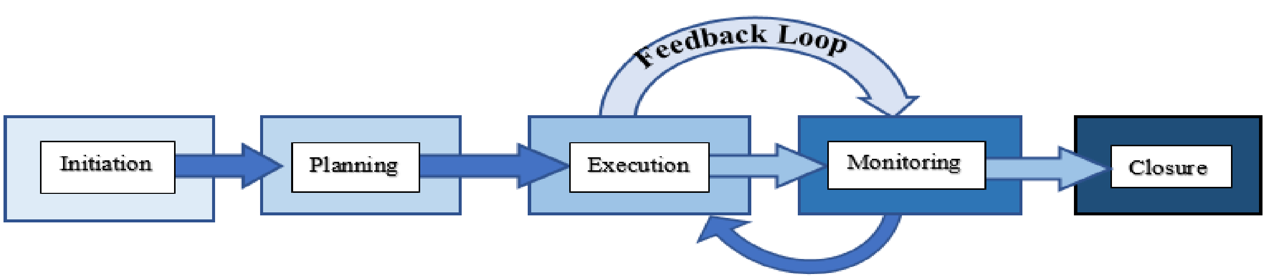


Figure 2: (DM Billing Metric Standardization Project's Life Stage) (Atlassian, 2023)(Personal collection, 2025)

4.3 Rational Reason for Applied Methodology:

DMCR billing standardization project approach circumnavigates between the values and principles set by the PMBOK, particularly tailoring project needs, deliverance of value to the project stakeholders, and curating systematic tasks for minimizing risks and challenges, which are as follow: “The Project Alignment with Principles of the PMBOK.” (Project Management Institute, 2021)

PMBOK Principle	PMBOK Principle Implementation
Tailoring	Employing a hybrid approach tailored to the combination of compliance and collaboration.
Systems Thinking	Collaboration of finance, Contracts and budget, system operations working together as a cohesive entity.
Stakeholder Engagement	Developing transparency by executing proper communication plan. Conducting feedback survey and workshops.
Value Delivery System	Less discrepancy in revenue for DMCR, improved cash flow, limited invoice rejections, and business case with compliance readiness will bolstered sponsor trust.
Quality	Focus on following the WCG IRB regulations guidelines to avoid rejections, getting DMCR’s AR billing model ready for audits, and accurate financial reporting.

Table 4: (PMBOK Principles implication on DM Billing Metric Standardization Project (Project Management Institute, 2021)

5 PROJECT BUISNESS CASE

5.1 Options Considered

The project manager at DMCR provided the executive board with definitive evidence and research indicating an urgent necessity for modifications in the CTA billing analysis. Additionally, the report outlined the potential risks and benefits for the company. This business case report was subsequently examined by the committee, which concluded that the project is essential for the company’s financial and reputational gain. The evaluation of these three options, leads to the selection of the final **option for “doing something more substantial”** for the betterment of billing process at DM Clinical Research.

Options	Potential Outcomes
To Do Nothing	This option considered to keep the existing billing system, that utilizes the financial data from CTA. This option will same DMCR time and team extra team efforts for forming a project team. But on the other hand it will cost DMCR millions of dollar in revenue in long run, due to using faulty billing system based on CTA vague language, that will create compliance and persistent financial discrepancies in the organization.
To Do Something	This option suggested to outsource a vendor to provide DMCR with a third party service for its AR billing data maintain and reconciliation. It is a fast and quick method and dose not disrupts the current process of DMCR finance department, however this option will cost a lot of money, involve signing a yearlong contractual subscription model and can pose a threat for data privacy breach.
To Do Something more substantial	The last option was to build an in-house billing tool, that will help DMCR with its CTA data to align with its AR billing processes and ensure compliance without any threat to its data privacy. Although this option asked DMCR to invest time, money and team efforts. Which will cause disruption to its ongoing clinical process and ask for a one-time investment, but in the long run this project will resolve all billing issues for DM Clinical Research and immediately provide huge returns on investment with in few months of project execution.

Table 5: (Options for selecting a project and benefit management analysis) (Bradly, 2016)

5.2 The Project Quality Triangle

The three primary constraints establish the foundation for the billing metric standardization project's business case at DMCR, a methodology also referred to as the "quality triangle" or "iron triangle." (Princes, 2019)

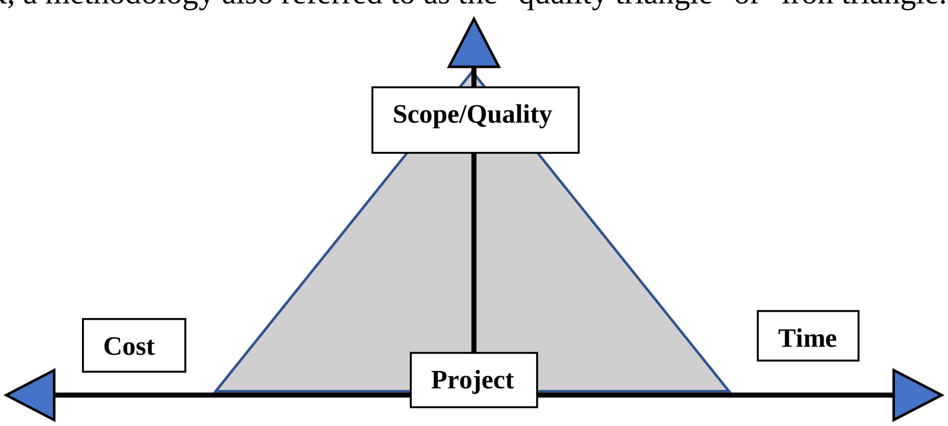


Figure 3: (The Three Constraints Project Iron-Triangle) (Wysocki, 2019)

Quality Triangle Element	Justification on Business Case
Scope	Clearing all financial uncertainty, establishes audit-compliant procedures, enhances sponsor confidence by improved invoice quality and transparency of procedures.
Time	Project to be delivered within six months in order to adhere the ongoing business and financial deadlines and avert additional inefficiencies and potential revenue missed.
Cost	Allocated resource for the project and cost to build the matrix is set at $320K - $350K Inclusive of (Core Team, recruitment, outsourcing of professional project tools, Contingency Buffer) Project to be deliver within this set budget. A controlled investment that is in line with the long-term financial and reputational gains of DM Clinical Research.

Table 6: (DMCR Project Iron-Triangle Constrains)

5.3 Associated Risks Involving Three Constrains

Project Scope Inflation: As the DMCR billing matrix follows an agile feedback loop at the project controlling stage, this may inadvertently invite more requests or unanticipated "enhancements," resulting in an increase of scope beyond the project's initial goals.

These enhancements are directly proposal to the time and cost constrains of the project, resulting delays in the timeline, excessive expense on added resource, and possible diversion from primary deliverables. In order to mitigate this risk, the project manager is responsible to hold weekly project scope review meetings with stakeholders involved in the project controlling phase, to keep the project framework intact as per the end goal. This helped to keep the project process and delivery simplified and saves the project teams to develop any complexity with in the project scope.

Low Quality Team Meeting: A research report by Ramserran and Haddud indicates that administering an online or hybrid project poses significant challenges for project managers, hence directly jeopardizing project quality (Ramserran & Haddud, 2018). The DMCR project team observed that daily online meetings, lasting 1 to 2 hours, exhibited low productivity levels. This resulted in delays in project task completion, as not all stakeholders were fully engaged due to poor internet connections and a lack of valuable contributions.

In order to mitigate this risk, the project manager implemented a new communication plan to enhance the project team’s motivation and productivity level, by setting a limit for reoccurring meetings to weekly and bi-weekly format. These weekly meeting were set for the team leads to discuss any hurdle or challenges faced in project task completion and the bi-weekly meeting were set to discuss the overall project review, the time limit of these meeting were set to an hour with 10 minutes of break in between, after taking consent these meetings were also recorded for those stakeholder who could not join in due to poor connection, after every meeting the project manager was responsible to take notes and deliver the minutes of the meeting by sending out an official email to all the stakeholders directly involved in this project along with the attached link for the recording of the meeting. This made sure a steady and valuable interaction with the project team, along with keeping every member filled in on the information about the project progress status and deliverable report.

6 ORGANIZTING TEAM STRUCHTURE

6.1 Roles and Responsibilities

Role	Individual Accountable	Responsibilities
Project Sponsor (P.S)	Zohar H. (CFO) DM Clinical Research	Facilitate project approvals, allocate financing, manage resources, and establish strategic direction.
Project Manager (P.M)	Claudia G. AR Project Manager	Oversees project implementation and documentation and stakeholder management. Accountable for project delivery.
Lead of Account Receivable (AR T.L)	Mamata D. AR Team Lead	Responsible for designing AR billing process alignment and training material.
Lead of System Operations (Sys Ops.)	Usama A. Clinical System Team Lead	Responsible for developing the AR Billing Matrix.
Contracts & Budgeting Lead	Taha L. Contracts Review Team Lead	Responsible for reviewing approved CTAs and making historical data for system.
AR Compliance Administrator (AR C.A)	Wali B. AR Compliance Admistrator	Submitting regulatory approvals and project compliance report to executive board.
Human Resource (HR)	Hussain K. Talent Accusation Team Lead	Resource and personnel management.
Project Advisor Board (PMO)	PMO (Project Advisor) CEO, Managers	Providing Project Support with risk mitigation and logistics.

Table 7: (DMCR B.M.S Project Team)

Table 6, titled "DM Clinical Research Billing Metric Standardization Project Team," describes the clearly defined roles for project execution. This was essential to eliminate any ambiguity for the project team and prevent adherence to a top-down hierarchy. Given that this project employs a decentralized governance model, it is crucial to identify the major functions of all team members to eliminate any delays in conveying significant developments about project progress.

6.2 Project RACI Matrix

Project Task	Project Team	P.M	P.S	P.M.O	A.R T.L	AR C.A	C & B	Sys Ops	HR
Project Execution and Completion		A	I	C	R	C	C	C	I
System Design		A	C	C	C	C	C	R	I
System Implementation		A	C	C	R	I	C	C	I
Compliance Review		A	C	C	I	R	C	I	I
Testing and Feedback		A	I	I	C	I	C	R	I
Training & Support		A	I	I	R	C	I	I	C
Approvals and Resource Allocation		R	A	C	I	I	I	I	C
CTA Historical Data Review		A	C	C	I	I	R	I	I

Responsible | Accountable | Consulted | Informed

Exhibit 2: a RACI matrix featuring a compilation of deliverables along with the designated responsible team for each one at DMCR for delivering a successful billing standardization matrix. (Matthews, 2024) (Personal Collection,2025)

7 COMMUNICATION PLAN

7.1 Communication Framework

The project manager at DMCR was faced with multiple issues when selecting a communication framework and assigning communication duties to the individual team members invoiced in the project. This was caused due to the project team's disconnectivity and geographical location challenge. To address this difficulty, an online model was chosen to facilitate communication effectively. (Eric , et al., 2014)

Each project teams were designated a communication level on the project governance framework, which helped in clearly establishing a communication network, this approach followed a side to side and top to down chain of commands, where each team is centrally connected to the project manager on the next level to request any sort of approval, raise concerns like potential delays, or asking for a strategic input. On the other hand these teams are also de-centrally connected with other team members on the same level, As each department works together for achieving the same goal, each team have an opportunity on the same level to raise concerns immediately if a project task faces a potential challenge, which can be then proactively assisted in resolving.

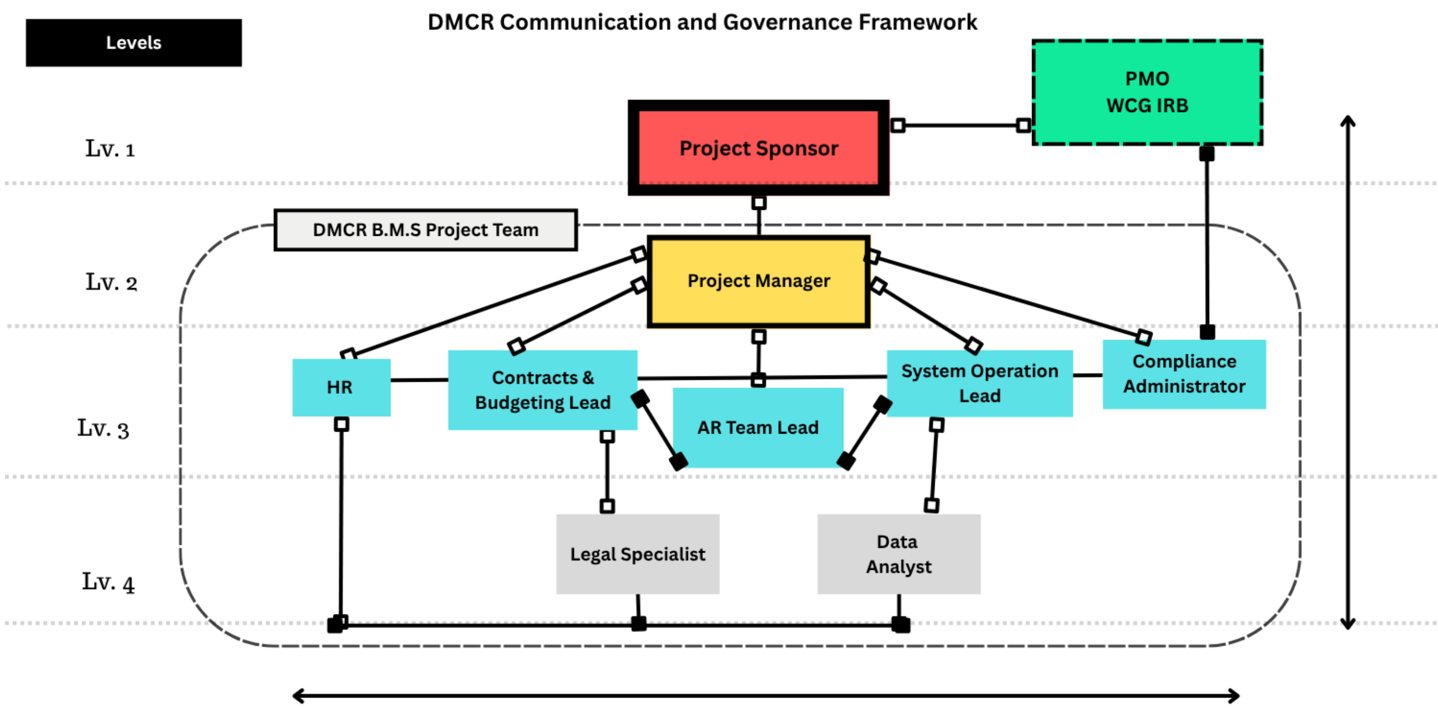


Figure 3: (DMCR Project's Communication and Governance Network) (Personal collection, 2025)

7.2 Selected Tools and Channels of Communication

DMCR spent in identifying the appropriate communication medium, evaluating each tool prior to adoption to ensure all teams are familiar with these tools. Here is a breakdown of all the tools and channels utilized on this project:

Communication Task	Tools / Channels	Justification
Project Management	ATOM, Power BI, Monday.com	Monitoring tasks, establishing milestones, and maintaining risk registers
Internal Stakeholders (Project Team Communication)	Google Team, Google Chat.	Immediate notification and prompt clarifications
Project Updates to External Stakeholders	Monthly Companies Newsletter, Email, Florence.	Version control, audit logs, policy documents, amendments updates.
Virtual Meeting and Training	Google Meet and Zoom, Google Calendar.	Weekly and bi-weekly Meeting scheduled, recording sessions, and updates.
Financial Reporting to Executive Board	Looker.IO, CRIO reports and Email.	Visualize accrual data, KPIs, and status updates

Table 8: (DMCR Project's Communication Channels and Tools) (Personal collection, 2025)

7.3 Consistency of Communication Delivery

The major issue that was initially faced by the project manager was the low quality of team meeting and motivation, this was measured after conducting a google survey among the project team members, as per the feedback that was collected, it was determined that the frequency of the meeting was effecting the project communication quality. To mitigate this issue PMO recommended a bi-weekly and weekly team meeting model to engage all the stakeholders and elevate projects quality in delivering communication. Here is the breakdown of the commination consistency plan that was implemented:

Communication Plan Stage	Recurrence Rate	Project Stakeholder	Communication Objective
Project Initiation	One Time	All Stakeholder	Project Team, Scope and Timeline
Weekly Project Progress Updates	Weekly	Project Sponsor, PMO, Finance, Clinical Ops	Project Status Reports, Risks, KPIs
Team Check-ins	Bi-Weekly	Core Project Team Members	Task Completion Status, Challenges, Quick Updates
Updates to Board of Directors	Monthly	Sponsor, Executive committee, and Project Manager	High-level Progress, Decisions and Approvals.
Risk and Conflict Escalation	As required	Management Committee and Relevant Project Team.	Mitigating Urgent Project risk and Conflicts.
Training Sessions	Once Per project Stage (ref. to Table 3.)	All Internal Stakeholders	Effective Change Management
Sponsor Communication	Monthly	External Sponsors / PMO	Project Feedback loops and Compliance Report
Project Close Out	One Time	All Stakeholder	Evaluate Results, Document Changes

Table 8: (DMCR Project's Communication Recurrence Chart) (Personal collection, 2025)

8 STAKEHOLDER MANAGEMENT

The internal stakeholders, consisting of the DM BMS team, reference to figure 3, participated actively in the project via monthly updates, workshops, technical meetings, and training sessions. The groups directly participated to the design, development, and implementation of the project plan, while the project Sponsor and PMO offered strategic supervision and approvals during monthly milestone reviews.

Moreover, the external stakeholders, including trail sponsor, were involved mainly for review and approval at start-up and close-out stage of the project. Whereas regulatory bodies and vendors were not engaged until policy changes directly affected the project parameters or billing item cost changes.

8.1 Identification Project Internal Stakeholders

Here is the list of stakeholders that were directly involved on DM Clinical Research account receivable billing standardization matrix project, these stakeholders are:

- Project Sponsor (DM Clinical Research Chief Finance Officer)
- Finance / AR Team (Accounts Receivable Manager and Team lead)
- Quality Assurance Team (Accounts Receivable Compliance Administrator)
- Contract and Budgeting (Contract Team Lead and Legal Specialist)
- Systems Operation (Data Team lead and Data Analysist)
- Human Resource Office (Talent Accusation Team Lead)
- PMO (DM Consultancy Board)

8.2 Identification Project External Stakeholders

The project manager has developed a list of stakeholders who were indirectly connected with DMCR in this project:

- Trail Sponsor (Moderna, Pfizer)
- Institutional Review Board (IRB)
- HIPPA (Government Regulatory Board)
- Vendors (Site Level Procurement Vendors)

9 PROJECT ACTION PLAN

9.1 BMS Work Breakdown Structure and Task Interdependencies:

WBS ID	Task Name	Description	Owner / Department	Dependencies
1	Project Kickoff	Initiate project, define objectives, confirm scope	Project Manager	None
2	Current State Assessment	Analyze existing accrual processes, identify gaps	Project Sponsor, PMO and PM	1
2.1	Resource Allocation	Analyzing current resources, outsource labor	PM and HR	2
2.2	Document Current Processes	Gather documentation from AR and Clinical Ops	PM and Compliance Administrator	2.1
3	Identify Pain Points	Review issues from past accruals, audits	AR Team, PMO	2.2
3.1	Accrual Model Design	Develop standard accrual matrix, draft SOPs	System Operation team, PMO and AR Team	3
3.2	Design Accrual Template	Create matrix for site-level costs	System Operation	3.1
4	Draft SOP Documentation	Prepare procedures and governance documents	Compliance, PMO	3.2
4.1	Stakeholder Validation	Gather feedback, validate model with stakeholders	PMO, Finance, Clinical Ops	4
5	Conduct Review Workshops	Collect feedback through meetings/workshops	PMO, AR Team	4.1
5.1	Pilot Testing	Test the new matrix with active studies	System Operation, AR	5
5.2	Conduct Testing & Feedback	Apply accrual matrix and gather results	Project Sponsor	5.1
6	System Integration	Integrate matrix into financial/clinical systems	System Operation, PMO, Compliance Administrator	5.2
6.1	Configure Financial Systems	Implement into AR tools, reporting dashboards	System Operation, AR team	6
6.2	Validate Integration	Test reporting outputs for accuracy	PM, PMO, Compliance Administrator	6.1
7	Training Delivery	Train internal teams on new processes	PMO, AR, Compliance	6.2
7.1	Develop Training Materials	Prepare guides, FAQs, and SOPs	PMO and Compliance Administrator	7
7.2	Conduct Training Sessions	Deliver workshops and Q&A	Compliance Administrator and AR team	7.1
8	Project Closeout	Final review, documentation, lessons learned	PM and PMO	7.2
8.1	Final Sign-off & Handover	Confirm completion, secure approvals	PMO, Sponsor	8
8.2	Document Lessons Learned	Capture insights for future improvements	Project Management Team and Project Sponsor	8.1

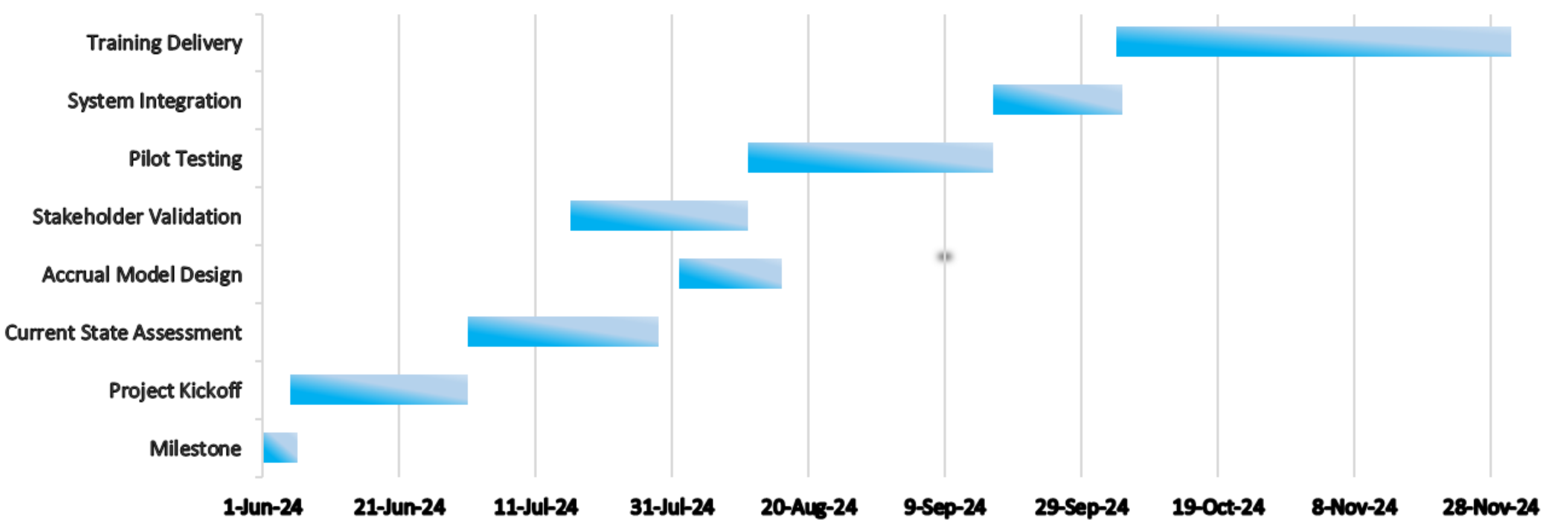
Table 8: (DMCR Project's WBS and Project Task Interdependency) (Personal collection, 2025)

9.2 Project Durations and Gantt Chart

Here is a comprehensive breakdown of all the key milestones and task associated with the DM BMS project delivery:

Milestone	Start Date	End Date	Duration (Days)
Project Kickoff	1-Jun-24	5-Jun-24	5
Current State Assessment	5-Jun-24	30-Jun-24	26
Accrual Model Design	1-Jul-24	28-Jul-24	28
Stakeholder Validation	1-Aug-24	15-Aug-24	15
Pilot Testing	16-Jul-24	10-Aug-24	26
System Integration	11-Aug-24	15-Sep-24	36
Training Delivery	16-Sep-24	4-Oct-24	19
Project Closeout	4-Oct-24	30-Nov-24	58

The project was completed within the given time frame successfully; this table shows the total number of days taken to complete each task by the project team at DM Clinical Research, here is the Gantt chart representation of the project execution:



Graph 1: (DMCR Project's Gantt Chart) (Personal collection, Excel 2025)

9.3 Cost and Resources Allocation

The overall projected expenditure for this project was sanctioned at $350,000 (estimated), and the duration of the project extended across six months. **Please be advised that the quantities are an estimate, as the actual cost has not been disclosed to the DMCR company policy**. (DMCR, 2025) The project manager created a cost and resource plan to successfully complete this project within the short timeframe of six months. Here is the detailed allocation of the budget, in this table we have specified the distribution of resources and the estimated cost of the project.

- **Project Team Expenses (Primary Project Expense):**

Role	Monthly Rate (USD)	Duration	Total (USD)
Project Manager	$8,000	6 months	$ 48,000.00
Finance AR Lead	$6,500	6 months	$ 39,000.00
System Operation Lead	$6,000	6 months	$ 36,000.00
Legal Specialist	$7,000	6 months	$ 42,000.00
AR Compliance Administrator	$6,500	6 months	$ 39,000.00
Data Analyst Support	$4,500	6 months	$ 27,000.00
Budgeting and Contract Lead	$6,000	6 months	$ 36,000.00
Total Project Team Cost For Six Month			**$ 267,000.00**

- **Subscription to Project Tools (Secondary Project Expenses)**

Subsriptions	Estimated (USD)
Project Management Tools (Monday.com, Power BI, ATOM)	$ 3,000.00
Financial Software / Reporting (Power BI, Looker.io)	$ 2,500.00
Compliance / Document Management (Florance, OneTrust)	$ 2,000.00
Total Estimated Spent on Tools and Technology	**$ 7,500.00**

- **Project Training and Workshops (Secondary Project Expenses)**

Activity	Estimated Cost (USD)
Internal Training Materials	$2,000
Workshops / Training Sessions	$3,000
Total	**$5,000**

- **Miscellaneous and Contingency Buffer (Contingency Reserve)**

Category	Estimated Cost (USD)
Buffer for Unforeseen Costs	$15,000
Miscellaneous	$15,000
Total	**$30,000**

- **Total Actual Budget Spent after Project Delivery:**

Category	Estimated Cost (USD)	
Personnel	$	267,000.00
Technology / Tools	$	7,500.00
Training / Change	$	5,000.00
Contingency and Misc.	$	3,000.00
Total Estimated	**$**	**282,500.00**

The actual projected cost after project delivery was around $282,500 over a duration of six months. This includes people, processes and technologies, training, getting stakeholders involved, and having a big enough safety net for any unforeseen circumstances. The Actual approved budget was set at $320,000 which guaranteed the allocation of resources necessary to achieve compliance, quality, and operational goals efficiently. (Parke, 2013) All of the necessary project requirement were met with in the total spent budget.

10 PROJECT CONTROLS

10.1 Project Decision Gates

From the initial stage of the project life cycle to the final project delivery and closure, the DMCR project of AR invoicing Metrix Standardization necessitated numerous decision-making steps. (Gupta, et al., 2017) Here is an analysis of these decision gates:

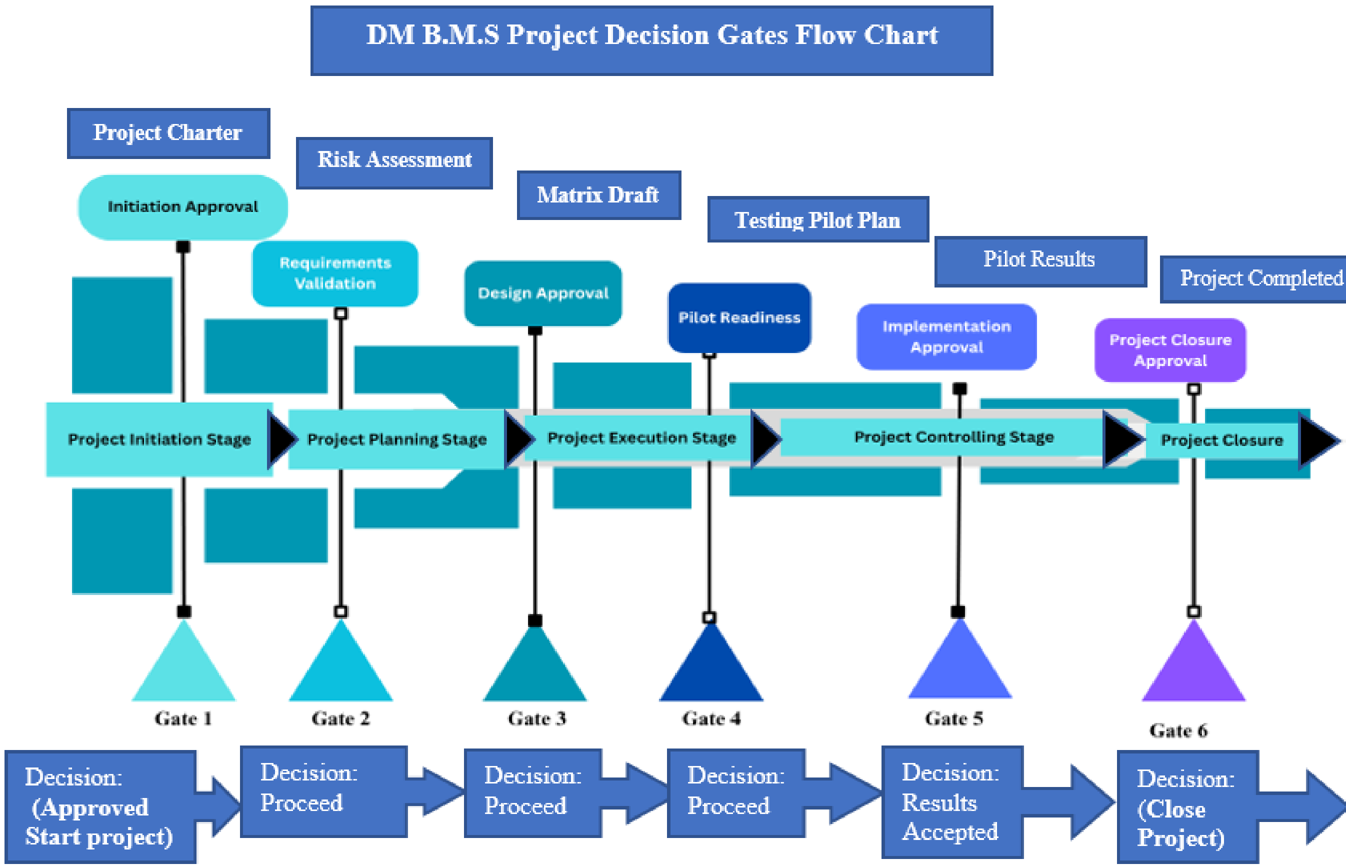


Figure 4: (DMCR Billing Matrix Standardization Project's Decision Flow Gate Chart) (Personal collection, 2025)

10.2 Progress Monitoring

Project processes were monitored using these key techniques:

Method	Frequency	Purpose
Gantt / Milestone Tracking	Ongoing	See that the plan is followed.
Team Lead Status Reports	Weekly/Biweekly	To Track Project Progress
CRIO Dashboards	Real-Time	Visual progress monitoring
Google Sheet Risk Logs	Ongoing	Proactive risk management
Executive Bored Review	Monthly	Governance

Table 9: (DM BMS Project Process Monitoring Methods) (Personal collection, 2025)

10.3 Conflict Management

The project was performed efficiently without inciting any possible conflicts among team members, since it lasted just six months and included a small team of nine. The project manager found it much more convenient to handle all concerns due to the decentralized project governance approach, (Eric , et al., 2014) which afforded everyone an equal chance to voice their issues at any stage of the project. Overall, the project was successfully delivered without any significant conflicts.

10.4 Lessons learned process

- **Enhanced Budget Forecasting Precision**: The preliminary budget may have been inflated owing to cautious assumptions, risk contingencies, or insufficient specific information during the planning phase.
  Future Action: Enhance budgeting methodologies by using the actuals from this project as a standard for analogous future endeavors. Concentrate on refining assumptions about staff expenditure, tool expenses, and contingency strategies. (Meredith & Shafer, 2022)

- **Efficient Use of Resources:** The lesson is that the project may have gone better if resources had been used more efficiently, tasks had been better prioritized, or the team had been more productive.
  Way forward: Write down what made these efficiencies possible. For example: include distinct responsibilities, minimal duplication of effort, and exceptional team participation. These attributes should be implemented in future resource planning models.

- **Risk Management:** It's possible that the risks were overstated, or that excellent risk management efforts kept the projected expenses from happening.
  Possible Action: Reassess the methodology for calculating risk buffers. It is advisable to modify contingency reserves in accordance with more precise, real-world data, rather than theoretical worst-case scenarios.

- **Managing Expectations and Communication with Stakeholders:** Clear communication and honest reporting may have helped prevent expensive shocks and extra labor.
  Next Steps: Keep putting stakeholder alignment and clear expectations at the top of the list of things that help manage the budget.

CONCLUSION

The Account Receivables Billing Matrix Standardization (B.M.S) Project was initiated by the Account Receivables Department at DM Clinical Research to identify the discrepancies and inaccuracies in the accrual of the CTA budget report, which had previously resulted in audit risks, payment delays, and strained sponsor relationships. This project, in accordance with DMCR objectives for achieving financial accuracy and billing method streamlining, effectively implemented a standardized, compliant, and auditable billing matrix.

The project used a hybrid project management strategy, integrating structured predictive methodologies with adaptable Agile practices, to guarantee precise scope control, punctual delivery, and active stakeholder involvement. The suggested approach encompasses a standardized billing matrix, interaction with current financial systems, explicit standard operating procedures, and focused training to guarantee long-term acceptance and sustainability. (Eric , et al., 2014)

The project was completed within the specified six-month period and well within the expected budget of $320K for a team of nine. The results have equipped the research institution for accurate financial oversight, reduced sponsor rejections of invoices, audit preparedness, and built trust among external stakeholders.

This project exemplifies DM Clinical Research's dedication to operational excellence, financial integrity, and regulatory compliance. The insights gained from this project will provide a standard for future enhancements in finance processes across clinical trial institutions.

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